INFORMATION TO KEEP WITH YOU

# My complete treatment handbook

## Entrectinib monotherapy · adult

Understand your treatment. Know what to expect. Keep your plan to hand.

**Treatment pattern**

**Ongoing**

Follow the prescribed daily plan

**Regimen source**

**NCCP 00702**

Version 2

Daily oral treatment without a rest week

## Inside your handbook


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**My name**

**Hospital / oncology team**

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

SACTCHECK · CHAPTER 01

# Understanding your treatment

Entrectinib monotherapy · adult · National Cancer Control Programme (NCCP) 00702

## How this medicine supports your treatment

Entrectinib aims to control a cancer with a matching gene fusion. It may help shrink or slow cancer, including some cancer in the brain. Your team explains the treatment aim, uncertainty and alternatives for your exact setting.

ROS1 / TRK TARGETED THERAPY

### Entrectinib

**Blocks abnormal ROS1 or TRK growth signals**

Matching gene fusion identified → Abnormal growth signalling blocked → Cancer growth may slow

Entrectinib inhibits proteins made by selected ROS1 or NTRK gene fusions. Blocking these signals can slow cancer growth. The relevant fusion, tumour setting and previous medicines determine eligibility; ROS1 and NTRK are different indications.

Report heart, thinking/memory, vision and bone symptoms early.

## Your treatment setting

### ROS1 and NTRK are different

This adult suite covers entrectinib alone for ROS1-positive advanced non-small-cell lung cancer (NSCLC) before any ROS1 inhibitor (00702a), or a separate NTRK gene-fusion solid-tumour indication with no previous NTRK inhibitor and no satisfactory treatment options (00702b). Molecular tests and Irish access criteria must be confirmed. The adult 600 mg schedule does not apply to children.

NCCP 00702 v2 · official regimen · EMA Rozlytrek product information · updated 27 March 2025 · HSE managed access protocols · ROS1 and NTRK

### How long?

The usual adult starting dose is 600 mg by mouth once daily, every day without a planned rest week. Use your prescribed dose. Treatment continues while helping and side effects remain acceptable; there is no fixed number of cycles.

NCCP 00702 v2 · official regimen

## Evidence depends on the gene fusion and tumour

Separate studies found tumour shrinkage and periods of cancer control in ROS1-positive lung cancer and NTRK fusion-positive tumours, including some brain metastases. These studies generally had no randomised comparison group; they do not establish an individual survival benefit or superiority over other targeted medicines.

These group results cannot predict your outcome; other treatment options, prior medicines and tumour type matter.

## When to get help

**Emergency: 112 / 999.** Severe chest pain, major breathing difficulty, collapse or stroke symptoms need emergency help.

**Contact your treating team promptly:** Call the oncology team urgently for new or worsening breathlessness, swelling, palpitations or fainting; marked confusion or memory change; fever, jaundice, severe diarrhoea or inability to drink. Report new visual disturbance and focal bone pain promptly. Follow your unit's temperature/contact instructions. Severe breathing difficulty, chest pain or collapse needs emergency help.

YOUR ONCOLOGY TEAM · DAYTIME

**Insert numbers here**

Acute Oncology Service

YOUR ONCOLOGY TEAM · OUT OF HOURS

**Insert numbers here**

Ask for medical oncology

EVENINGS / OVERNIGHT

**Your agreed out-of-hours pathway**

Other hospitals: use the contacts your own team provides.

SACTCHECK · CHAPTER 02

# Your schedule, tests and appointments

Entrectinib monotherapy · adult · NCCP 00702

BASELINE → TREATMENT → FOLLOW-UP

## Treatment and checks over time

Relative milestones, not booked appointments or equal time periods. electrocardiogram (ECG)/ electrolytes are checked after one month; daily dosing has no planned rest week.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00702 v2 · official regimen · EMA Rozlytrek product information · updated 27 March 2025

THE WHOLE TREATMENT JOURNEY

## Entrectinib monotherapy · adult: treatment and checks

**Your treatment pattern**

Daily oral treatment without a rest week

This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

### ROS1-positive advanced NSCLC or separate eligible NTRK fusion-positive tumour

**Read from left to right.** Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

**Treatment, tests and reviews across the course**

| Care / check | Before treatment | First month | Ongoing daily treatment | Benefit / toxicity review | Hold or change |
|---|---|---|---|---|---|
| **Treatment** NCCP 00702 · SMPC | Confirm ROS1 or NTRK setting and prescription | Daily capsules without a rest week | Continue prescribed dose if authorised | Review benefit and tolerability | Team authorises hold, restart, switch or stop |
| **Blood tests** NCCP 00702 · SMPC | Blood count, kidney/liver tests and uric acid | Repeat at agreed reviews | Continue regular testing | Extra tests for symptoms/ abnormal results | Follow recovery and unresolved effects |

| Care / check | Before treatment | First month | Ongoing daily treatment | Benefit / toxicity review | Hold or change |
|---|---|---|---|---|---|
| **Heart checks** NCCP 00702 · SMPC | ECG/ electrolytes; left ventricular ejection fraction (heart pumping measurement ) (LVEF) if symptoms/ risk | ECG/ electrolytes after one month | Further checks periodically as indicated | Assess new breathlessnes s/swelling/ fainting | Confirm recovery before restarting |
| **Symptoms and supply** NCCP 00702 · SMPC | Cognition, vision, neuropathy and medicine list | Review symptoms, adherence and supply | Report thinking/ memory, visual or bone changes | Review interactions and practical support | Continue contact and reproductive precautions |
| **Cancer assessment** NCCP 00702 · SMPC | Validated fusion, stage and prior therapy | Clinical review | Scans at Consultant-directed intervals | Discuss response/ progression and preferences | Agree next treatment or follow-up |

## What each check is for

1

### Confirm the setting

Before starting

The fusion and previous inhibitor change eligibility.

**Ask** / **act:** Confirm validated testing, MAP approval, prescription and baseline checks.

2

### One-month heart review

After one month

QT and electrolytes can change without symptoms.

**Ask** / **act:** Agree who books the ECG/blood tests and reviews the results.

3

### Ongoing review

During daily therapy

Benefit, symptoms and organ function can change.

**Ask** / **act:** Keep the medicine list current and report symptoms between visits.

4 **Hold or change**

When toxicity/progression requires

Recovery and restart differ by adverse effect.

**Ask** / **act:** Follow the written clinician plan; continue follow-up and contraception advice.

**At the next review, ask**

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

## Before and during treatment

Regular checks help find changes early.

### Before the first dose

Before starting: confirm fusion, previous targeted treatment, access and pregnancy precautions; blood count, kidney/liver tests, uric acid, ECG/electrolytes and cognitive symptoms. Heart-pumping assessment is needed for heart-failure symptoms or risk; brain imaging follows the Consultant plan.

### Ongoing reviews

During treatment: repeat blood count, kidney/liver tests and uric acid at agreed reviews. Repeat ECG/electrolytes after one month, then periodically as indicated. Assess thinking/memory, heart symptoms, vision and bone pain; scans follow the Consultant plan.

## Practical treatment details

### Supply and appointments

Arrange repeat prescriptions before capsules run out.

Oral treatment needs no routine infusion line or pump.

SACTCHECK · CHAPTER 03

# Symptoms and supportive care

Entrectinib monotherapy · adult · NCCP 00702

### Daily capsules and planned reviews

No dates or personal information are needed. Confirm actual appointments with your team.

| Treatment or check | Timing in this view |
|---|---|
| **Preparation** | **Before treatment**<br>Confirm fusion, indication, access, blood tests and electrocardiogram (ECG). |
| **Daily treatment** | **First month → Ongoing daily treatment → Benefit / toxicity review**<br>Prescribed entrectinib every day; no planned rest week. |
| **Heart and blood checks** | **First month → Ongoing daily treatment → Benefit / toxicity review**<br>ECG/electrolytes after one month, then periodically as indicated; regular blood and symptom reviews. |
| **Hold or change** | **Hold or change**<br>Source-specific recovery and clinician decision before restart. |

Continue only as prescribed. A symptom or abnormal test can change treatment at any time.

Columns show sequence, not equal periods; the map does not authorise a restart.

> Columns show sequence, not equal durations. These are reference milestones, not booked appointments. Symptom bands do not predict severity or a safe time to ignore symptoms.

## Taking entrectinib

Follow your own prescription.

### Whole capsules and missed doses

Take whole capsules with or without food; do not crush or chew. Avoid grapefruit, grapefruit products and Seville oranges. If a dose is missed, take it unless the next dose is due within 12 hours; confirm the exact boundary with your pharmacist. The leaflet permits repeating whole capsules after immediate vomiting: follow your written plan and call if uncertain. Suspension or tube administration requires separate pharmacy instructions.

### Interactions

Give the team a complete medicine and supplement list, including St John’s wort. CYP3A inhibitors/inducers and medicines affecting heart rhythm need pharmacy review. Do not change the cancer dose yourself; temporary interaction doses differ from toxicity reductions.

### Swallowing difficulty

Ask pharmacy for the approved method. Current SmPC allows capsule suspension and enteral administration; the older NCCP says not to open capsules. Instructions and vomiting advice depend on the formulation.

## Living with treatment

Report changes between visits.

### Thinking, vision and driving

Do not drive or use machinery while experiencing dizziness, fainting, blurred vision or cognitive effects. Ask family/supporters to report noticeable memory or behaviour changes.

### Bones and falls

New focal pain, changed gait or mobility needs prompt review for a fracture; reduce fall hazards where possible.

### Pregnancy, fertility and breastfeeding

Avoid pregnancy. Women who could become pregnant need highly effective contraception during treatment and for at least five weeks afterwards; add a barrier method if using systemic hormonal contraception. Men with partners who could become pregnant need highly effective contraception during treatment and for at least three months afterwards. Do not breastfeed during treatment. Discuss fertility and family plans before starting.

### Support medicines

Use prescribed symptom treatment. No routine steroid, granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) or antimicrobial prophylaxis is specified for this monotherapy; avoid borrowing an infusion antiemetic schedule.

NCCP 00702 v2 · official regimen · NCCP systemic anticancer therapy (SACT) antiemetic guidance V6 · oral therapy · NCCP NCIS antiemetics V8 · parenteral scope

## When to get help

Do not wait for the next appointment.

### Call urgently

Call the oncology team urgently for new or worsening breathlessness, swelling, palpitations or fainting; marked confusion or memory change; fever, jaundice, severe diarrhoea or inability to drink. Report new visual disturbance and focal bone pain promptly. Follow your unit's temperature/contact instructions. Severe breathing difficulty, chest pain or collapse needs emergency help.

## What studies can tell us

### Your decision

Ask how the correct molecular and tumour-specific study population compares with your situation. Study averages do not establish personal prognosis or rank current therapies.

Fan et al. 2024 · ROS1 integrated analysis · Demetri et al. 2022 · adult NTRK integrated analysis · Cho et al. 2024 · NTRK-positive NSCLC analysis

## When treatment changes

Continuation and restart are clinical decisions.

### Hold or stop

Ask why treatment is held, which tests or symptoms must recover, who will authorise restarting and what follow-up is needed.

## Practical support

### Symptom medicines

NCCP rates entrectinib minimal-to-low oral emetic risk. Agree individual rescue treatment and check heart-rhythm interactions; no routine infusion premedication is specified.

### Daily life and medicines

Keep the medicine list current. Avoid driving or machinery while dizzy, faint, visually affected or cognitively affected. Review falls risk and report bone pain.

### Pregnancy and breastfeeding

Use the agreed contraception plan during and after treatment. Do not breastfeed during treatment; ask about fertility and family plans before starting.

Taking tablets / attending treatment

## Questions to ask

#### Why do I need an ECG?

Entrectinib can affect heart rhythm. ECG/electrolytes are checked before treatment, after one month and periodically as indicated; new fainting or palpitations needs urgent advice.

### Can I drive?

Do not drive or use machinery while dizzy, faint, cognitively affected or visually affected. Ask your team when it is safe to resume.

### Do I need to enter dates?

No. The timeline shows relative stages. The team books actual tests and reviews the results.

**Explain the plan back:** Explain your daily dose, what to do after a missed or vomited dose, which symptoms need a call, and who arranges your one-month ECG/electrolytes.

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**Adult treatment scope · NCCP 00702a / 00702b** This adult suite covers entrectinib alone for ROS1-positive advanced non-small-cell lung cancer (NSCLC) before any ROS1 inhibitor (00702a), or a separate NTRK gene-fusion solid-tumour indication with no previous NTRK inhibitor and no satisfactory treatment options (00702b). Molecular tests and Irish access criteria must be confirmed. The adult 600 mg schedule does not apply to children.

**Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

## Continuous entrectinib · individual support

**Active medicines:** entrectinib

**Emetogenic risk:** Minimal to low (oral)

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

### Additional phase-specific support

- NCCP minimal-to-low oral emetic risk: agree individual rescue treatment and check QT/interactions. NCIS V8 parenteral schedules are not a daily oral prescription.
- No routine premedication, granulocyte colony-stimulating factor (white-cell growth support) (G-CSF), steroid course or antimicrobial prophylaxis is specified.
- Give the team a complete medicine and supplement list, including St John's wort. CYP3A inhibitors/inducers and medicines affecting heart rhythm need pharmacy review. Do not change the cancer dose yourself; temporary interaction doses differ from toxicity reductions.
- Take whole capsules with or without food; do not crush or chew. Avoid grapefruit, grapefruit products and Seville oranges. If a dose is missed, take it unless the next dose is due within 12 hours; confirm the exact boundary with your pharmacist. The leaflet permits repeating whole capsules after immediate vomiting: follow your written plan and call if uncertain. Suspension or tube administration requires separate pharmacy instructions.
- Avoid pregnancy. Women who could become pregnant need highly effective contraception during treatment and for at least five weeks afterwards; add a barrier method if using systemic hormonal contraception. Men with partners who could become pregnant need highly effective contraception during treatment and for at least three months afterwards. Do not breastfeed during treatment. Discuss fertility and family plans before starting.
- Review cognition/driving, falls/fractures, uric acid, swelling, visual symptoms and practical adherence needs.

**Before signing:** verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

**Adult treatment scope · NCCP 00702a / 00702b** This adult suite covers entrectinib alone for ROS1-positive advanced NSCLC before any ROS1 inhibitor (00702a), or a separate NTRK gene-fusion solid-tumour indication with no previous NTRK inhibitor and no satisfactory treatment options (00702b). Molecular tests and Irish access criteria must be confirmed. The adult 600 mg schedule does not apply to children.

NCCP 00702 · VERSION 2

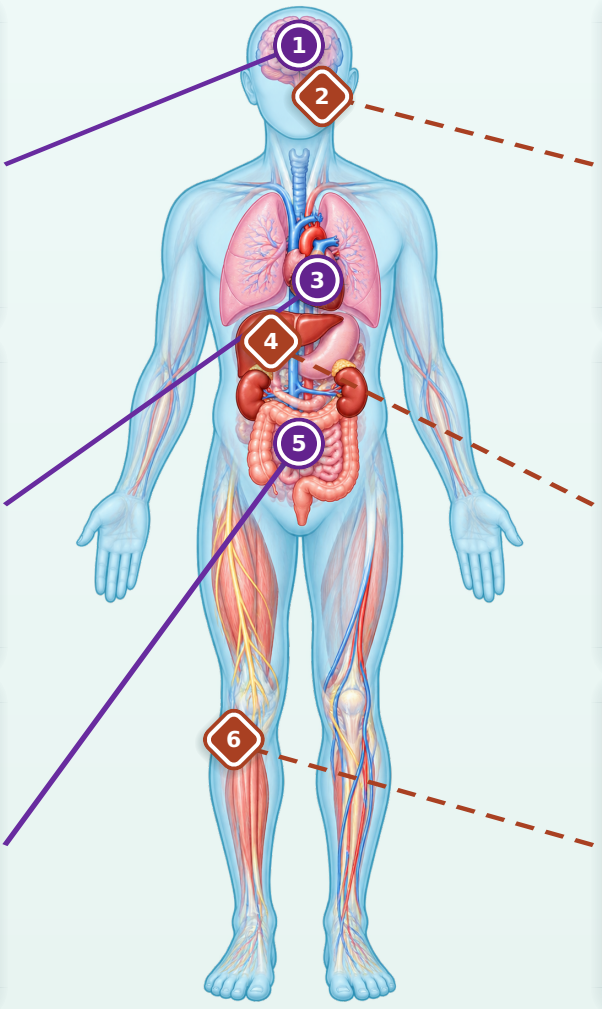
# Entrectinib monotherapy · adult

The usual adult starting dose is 600 mg by mouth once daily, every day without a planned rest week. Use your prescribed dose. Treatment continues while helping and side effects remain acceptable; there is no fixed number of cycles.

Oral targeted therapy · selected medicine-specific effects. Full details and sources are in the complete reference.

## A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.



### 1 Dizziness / thinking changes

**entrectinib**

Report new confusion, memory or balance problems; avoid driving while affected.

### 2 Visual changes

**entrectinib**

New blurred vision or other visual disturbance needs review.

### 3 Breathlessness / swelling / fainting

**entrectinib**

Heart pumping or rhythm problems require urgent assessment.

### 4 Jaundice / liver tests

**entrectinib**

Liver injury may need blood tests and interruption.

### 5 Bowel symptoms / nausea

**entrectinib**

Report severe symptoms or dehydration promptly.

### 6 Bone pain / altered gait

**entrectinib**

Possible fracture needs prompt assessment.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Call the oncology team urgently for new or worsening breathlessness, swelling, palpitations or fainting; marked confusion or memory change; fever, jaundice, severe diarrhoea or inability to drink. Report new visual disturbance and focal bone pain promptly. Follow your unit’s temperature/contact instructions. Severe breathing difficulty, chest pain or collapse needs emergency help. Severe chest pain, major breathing difficulty, collapse or stroke symptoms: 112 / 999.

SACTCHECK · CHAPTER 04

# Sources and further information

Entrectinib monotherapy · adult · National Cancer Control Programme (NCCP) 00702

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.


1. **NCCP 00702 v2 · official regimen**
   https://healthservice.hse.ie/documents/6770/702_V2_Entrectinib_monotherapy.pdf
2. **EMA Rozlytrek product information · updated 27 March 2025**
   https://www.ema.europa.eu/en/documents/product-information/rozlytrek-epar-product-information_en.pdf
3. **HSE managed access protocols · ROS1 and NTRK**
   https://healthservice.hse.ie/staff/information-healthcare-workers/medicines-management-programme-mmp/managed-access-protocols/
4. **Fan et al. 2024 · ROS1 integrated analysis**
   https://pmc.ncbi.nlm.nih.gov/articles/PMC11733145/
5. **Demetri et al. 2022 · adult NTRK integrated analysis**
   https://pmc.ncbi.nlm.nih.gov/articles/PMC9365368/
6. **Cho et al. 2024 · NTRK-positive non-small-cell lung cancer (NSCLC) analysis**
   https://pubmed.ncbi.nlm.nih.gov/38171156/
7. **NCCP NCIS antiemetics V8 · parenteral scope**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf
8. **NCCP systemic anticancer therapy (SACT) antiemetic guidance V6 · oral therapy**
   https://healthservice.hse.ie/documents/7152/Classification_Document_for_Systemic_Anti-_Cancer_Therapy_SACT_Induced_Nausea__pWgDCrm.pdf